Nephrology Referral Criteria

Synthetic reference for demo purposes — general guidance, not a clinical guideline or St. Luke's policy.

Consider referral to nephrology when any of the following are present:

- **eGFR < 30** mL/min/1.73m² (stage G4 or G5) — refer to plan for kidney replacement therapy.
- **Rapid progression** — a sustained decline in eGFR of more than 5 mL/min/1.73m² per year, or a drop of ≥25% from baseline.
- **Severe albuminuria** — urine ACR > 300 mg/g (A3), or albuminuria with declining eGFR.
- **CKD with resistant hypertension** despite four or more agents.
- **Recurrent or extensive kidney stones**, or a suspected hereditary kidney disease.
- **Persistent abnormalities** — unexplained hematuria, electrolyte disturbances, or anemia of CKD.

Priority

Patients at stage G4/G5 who are **not currently seeing nephrology** are the highest-priority referrals. Early referral improves outcomes and allows time to prepare for advanced-stage care.